

# TRANSFORMING HEALTHCARE

*A Five-Pillar Framework with Vermont as the National Proving Ground*

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Revised Master Edition — Fall 2026 Editorial Audit

## EDITORIAL AUDIT & CHANGELOG SUMMARY

- **AHEAD Model Resolution:** Disambiguated federal CMMI AHEAD framework (active for national/other states like MD, CT, HI, RI, NY) from Vermont's context. Explicitly updated Vermont's trajectory to reflect its July 2026 formal withdrawal following CMMI's EAST Fund reduction (capped at ~\$10M vs. \$138M projected). Frame all hospital global budgets and reference-based pricing (RBP) strictly under Vermont's state statutory mandate (Act 68 of 2025). Replaced legacy phrasing like 'first AHEAD performance year' with 'first Act 68 global budget performance year'.
- **Temporal Anchor Fixed to Fall 2026:** Updated narrative present tense across Preface, Introduction, and Chapter 1 to Fall 2026. Events through Summer 2026 (Act 167 in 2022, Act 68 in 2025, \$195M RHT award in Dec 2025, AHEAD withdrawal in July 2026) are now consistently framed in the past tense.
- **Financial Standardization (\$138M Baseline):** Standardized all pre-cap EAST Fund projection references to \$138 million (correcting the \$150M typo previously in Figure 1.6).
- **Comprehensive URL & Cross-Reference Audit:** Sanitized malformed URLs (removing errant embedded spaces like 'p olicy' -> 'policy', 'apm-design' space fixes), converted all relative paths (/vermont-act-167, /chat, etc.) to full absolute links (healthtransformationreview.org/...), and unified ecosystem domain references under healthtransformationreview.org.
- **Pillar Dependencies & Logical Enhancements:** Added clarifying callout/sidebar addressing the Economics -> Operations capital vs. operating liquidity distinction (\$195M RHT statutory capital from Policy vs. operating margin from Economics), clarified Economics-as-Design (parallel stage 2) vs. Economics-as-Management (gated stage 3), and reinforced executive operational accountability for the Equity Imperative.
- **Visual & Structural Enhancements:** Embedded the high-resolution Five-Pillar & Dependency Matrix Architecture Diagram into Chapter 1.4, formatted worked example callout boxes, and polished table formatting throughout.

## PREFACE

This book began with a room full of people who already knew something was wrong.

In September 2024, the Green Mountain Care Board (GMCB) of Vermont convened the public meeting — recounted in detail in the Introduction that follows — at which the consulting firm Oliver Wyman presented the findings of a year-long, \$1 million statewide hospital systems analysis. The people in that room had come because they suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong. Nine of Vermont's fourteen hospitals were already operating at a loss. Thirteen of fourteen would be in the red by 2028, absent structural intervention. The cumulative five-year system deficit ranged from \$700 million to \$2.4 billion. Premiums had risen 108% in six years while household income grew 22%. The working-age population that funds the cross-subsidy keeping rural hospitals alive was declining. The over-65 population was growing toward 30% of the state.

That room is the origin of this book. Not because Vermont is uniquely troubled — every American state is facing some version of the same forces on a time delay — but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that thirty years of voluntary payment reform have not changed the fundamental incentives of the system. And early, therefore, in its willingness to do something that no state has done with this level of statutory specificity and capital commitment: mandate structural change.

This book is the analytical framework for that change. It is not a policy brief for or against any particular agenda. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested. This book engages that contestation honestly and presents both sides where the evidence genuinely runs in two directions.

The framework is organized around five pillars: Policy, Technology, Economics, Clinical, and Operations. It emerged from the conviction that transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural investment produce aspirations without outcomes.

That last failure is why equity does not appear here as a sixth pillar. It is the Equity Imperative — a single justice test that each of the five pillars must pass on its own terms. The Introduction explains why that distinction is structural rather than a matter of emphasis, and Chapter 1 applies the test pillar by pillar.

**The five pillars must move together — which means the people responsible for each pillar must understand all the others.**

Vermont runs through this book as its primary teaching case. Vermont is in this book not because it is large or powerful, but because it is transparent. The combination of a small state's intimacy, a strong

public-sector analytical tradition, and an active regulatory body in the legislature, the GMCB, and the Agency of Human Services (AHS) has produced a documentary record — the multiple legislative acts, the Oliver Wyman analysis, the GMCB’s monthly hospital data, the AHS transformation reports — that gives this book a foundation most states cannot match. Vermont’s transformation is happening in public, with detailed public data, and the lessons transfer.

This book is also not a prediction. The transformations described here are underway but unfinished, and their outcome is genuinely uncertain — the Introduction’s “What This Book Cannot Do” section says plainly what this book does and does not claim to know. What this book provides is the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

Finally, this book is not, on its own, the whole of the argument. This book is the intellectual foundation of the Health Transformation Review, an open-source integrated ecosystem with three parts designed to work as one:

1. The book (<https://healthtransformationreview.org/book>) you are reading — the framework, the dependency logic, the Vermont evidence, and the national comparisons.
2. The platform (<https://healthtransformationreview.org>) — five pillar hubs, a Research Lab of nearly forty interactive analytical tools, an AI Analyst grounded in this book and Vermont’s public-domain source documents, a fifty-state dashboard, and a daily intelligence feed. Every argument in these pages that can be modeled is modeled there.
3. The Academy (<https://healthtransformationreview.org/academy>) — a structured executive-education layer of courses, tracks, and lessons that teach the concepts hands-on and link directly to the tools that apply them.

The three are deliberately cross-linked. Each chapter ends with two doorways: an “Explore on the Platform” guide that maps the chapter’s argument to the specific tools that let you manipulate it, and a “Go Deeper — Academy” guide pointing to the lessons that teach it hands-on. The platform’s tools carry “From the Book” callouts back to the chapters that explain them; and the AI Analyst can answer a question and route you to the right tool or lesson in the same breath. Everything in the ecosystem is, in effect, an interactive expression of this book’s framework. You can read the book alone and it stands on its own — but the framework is meant to be run, not just read, and the platform is where you run it.

### EXPLORE ON THE PLATFORM

The full ecosystem is at <https://healthtransformationreview.org> — read the book online with audio narration at <https://healthtransformationreview.org/book>, open the Research Lab, ask the AI Analyst, or start a course in the Academy at <https://healthtransformationreview.org/academy>.

## INTRODUCTION

**September 18, 2024**

The room was full before the presentation started.

The Green Mountain Care Board of Vermont had convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group would present findings from a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The analysis had not been produced in isolation: over the preceding year, Oliver Wyman had run more than 230 meetings across all 14 of Vermont's Hospital Service Areas, engaging over 3,100 participants from more than 100 organizations — hospital leaders, state legislators and agency officials, the Vermont Hospital Association, health economists, community advocates, and clinicians. Many of the people who had taken part in that engagement now watched its conclusions arrive. They had come because they already suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong.

Oliver Wyman's lead consultant opened with the financial trajectory. Nine of Vermont's fourteen hospitals were already reporting operating losses, with the worst reaching negative 8.9%. If current trends continued — the same trends that had been building for a decade, the trends that voluntary reform had been addressing for years without reversing — thirteen of the fourteen would be in operating losses by 2028. The cumulative five-year system deficit ranged from \$700 million in the optimistic scenario to \$2.4 billion in the realistic one.

The next slide showed premiums. The average silver exchange plan premium had risen from \$456 per month in 2018 to \$948 in 2024 — a 108% increase. Median household income had grown 22% over the same period. Out-of-pocket maximums had more than doubled. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, was charging commercial payers roughly 358% of Medicare rates — RAND's published figure for inpatient and outpatient services combined, drawn from 2018–2020 claims — making it, by RAND's own accounting, the most expensive hospital in Vermont. Statewide, Vermont hospitals charge commercial payers 250–300% of Medicare on average, and the Green Mountain Care Board's own later analysis found outpatient imaging at some Vermont hospitals reaching 715–944% of Medicare. Those charges flowed directly into the premiums that Vermont families were paying, with no regulatory brakes.

The final slide showed the demographics. Vermont's working-age population was projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while the over-65 population rose from 21.7% toward 30% of the state's total. This shift would progressively move patients from commercial insurance, which pays above cost and funds the cross-subsidy that sustains hospitals, into Medicare and Medicaid, both of which reimburse below cost. The slide did not say the words "system collapse." It did not need to.

### **Vermont Is Not an Outlier. It Is a Preview.**

No one in that room disputed the numbers. No one argued that the trend would reverse itself. The question that followed — the question this book is an attempt to answer — was whether structural

change was possible. Not aspirational change. Not incremental improvement. But actual change to the fundamental incentives, relationships, and capabilities of the system. Whether a small rural state could force a healthcare transformation that the national system, with all its resources and political energy, had been deferring for decades.

Vermont's answer, enacted in Acts 167, 51, and 68 and supported by federal transformation capital, is the case study used in this book. Whether it constitutes successful transformation will be determined over the next five to ten years. What is already clear is that Vermont has committed to structural change with more specificity, more statutory force, and more capital investment than any comparably sized system in recent American health policy history.

This book is written in real time, closely watching that transformation. Not as a retrospective account of a completed success, but as an analytical framework for a transformation in progress — with all the uncertainty, potentially incomplete public information, and instructiveness that implies.

### **What Transformation Actually Means**

Few words in healthcare are used more often than “reform” or “transformation,” and the frequency has cost them precision. Both get applied to routine process improvements and to genuine structural change alike, and to aspiration as readily as to commitment — which makes it worth stating, at the outset, exactly what this book means by transformation.

It uses “transformation” in a precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program (\$195 million annually) that builds the community infrastructure that payment reform requires to produce population health improvement rather than just cost reduction.

### **How to Read This Book If Vermont Isn't Your Context**

This book uses Vermont as its primary teaching case for the reasons explained above: transparency, documentation, and an early position on a curve every state is on. But the subject of this book is the five-pillar framework — Policy, Technology, Economics, Clinical, and Operations — and the dependency

logic that links them. Vermont is the evidence, not the scope. If Vermont is not your context, here is how to get the most out of the chapters that follow.

**Translate the institutions, not just the policies.** Every named Vermont statute or institution in this book is a specific instance of a general category that exists, in some form, in every state. The Green Mountain Care Board is Vermont's version of a hospital rate-setting or cost-growth-benchmark authority — Massachusetts, Rhode Island, and several other states have analogous bodies under different names. Acts 167 and 68 are Vermont's versions of a diagnostic mandate and an operational mandate — the categories, not the bill numbers, are what transfer. AHS is Vermont's version of a state health and human services agency acting as a system operator. As you read, ask: what is the equivalent institution, statute, or agreement in my state — even if it is smaller, newer, less formal, or does not yet exist?

**Look for the framework statements, not only the Vermont data.** Each chapter makes a claim about how the pillars depend on one another that does not require Vermont to be true — for example, that technology investment must precede payment reform because payment reform without data infrastructure cannot be measured or trusted. Vermont's experience is the proof, presented in detail because detail is what makes an argument verifiable rather than asserted. But the claim itself is the point.

**Use the “Implications for You” sections as your entry point.** Most chapters close by addressing different reader roles directly — hospital executives, payers, state officials, clinicians. These sections are written to apply regardless of any state context, and are often the fastest way to find the actionable content inside a Vermont-heavy chapter. If you read nothing else in a chapter, read that section, then go back for the supporting detail you need.

In short: treat Vermont the way a business textbook treats a case study. The case is specific so that the lesson does not have to be.

**Vermont is not the audience for this book. Vermont is the evidence.**

## **The Strongest Case Against This Book's Argument**

This book argues that structural, mandatory reform can succeed where voluntary reform has failed. That is a contested claim, and intellectual honesty requires engaging the strongest version of the opposition before asking the reader to proceed on that premise.

### **3. The Federal Model Reliability and Durability Argument**

CMMI has operated innovation models for fifteen years. Most have not produced the savings they projected. AHEAD's predecessor — the Vermont All-Payer ACO Model — produced modest results over nine years despite significant federal and state investment. Why will this time be any different?

This is perhaps the strongest challenge in the book, and the honest answer begins by conceding the premise: CMMI models have generally underperformed their projections, and several have been redesigned or discontinued mid-course. What is different about Vermont's approach is the mandatory statutory architecture its predecessor lacked. Acts 167 and 68 require hospitals to participate in global budgets and reference-based pricing regardless of what happens federally. Vermont's transformation does not depend on AHEAD; AHEAD was to amplify it.

This ceased to be hypothetical in July 2026, when Vermont formally withdrew from AHEAD after a CMS renegotiation that cut Vermont's expected EAST Fund from roughly \$138 million to a cap near \$10 million. The mandatory state framework under Acts 167 and 68 remained fully in force, and the reform continued without missing a step. The skeptic's question was answered by events — Vermont did not need AHEAD to survive the loss of AHEAD. Chapter 13 (The Future of Healthcare Transformation) returns to this directly.

#### 4. The Oliver Wyman Independence Question

This book relies heavily on Oliver Wyman's Act 167 analysis. Oliver Wyman was paid \$1 million by the Vermont state government to produce a report that would support the case for reform. That is not independence. Readers should note that the Oliver Wyman financial projections — hospital deficit trajectories, administrative cost benchmarks, workforce shortage estimates — cannot all be independently verified from public sources, and the firm had a client relationship that may have shaped how it framed its conclusions.

This book treats Oliver Wyman's quantitative findings as estimates, not certainties, and notes throughout where specific figures cannot be independently confirmed. The structural argument — that Vermont's system faces a genuine financial crisis requiring structural response — is supported by public GMCB data, independent KFF analysis, and GMCB's own enforcement actions that do not depend on Oliver Wyman's projections.

#### The Five-Pillar Framework and the Equity Imperative: An Integrated Architecture

The central analytical contribution of this book is the five-pillar framework for healthcare transformation: Policy, Technology, Economics, Clinical, and Operations. It is not a checklist of things that should be done, but an integrated architecture for understanding why transformation attempts underperform when any pillar is addressed in isolation. When a pillar is missing or disconnected, the transformation cannot deliver its intended result: payment reform without the data infrastructure to manage it, or the clinical models to respond to it, produces financial pressure without improvement.

**Each pillar is defined by a single diagnostic question. Policy asks whether a reform is permissible. Technology asks whether it is possible. Economics asks whether it is sustainable. Clinical asks whether it is effective. Operations asks whether it is executable. An honest “no” to any one of them puts the transformation at risk, regardless of how confidently the other four answer yes.**

There is a sixth question, and it is deliberately not a sixth pillar: is it just? This is the Equity Imperative — a cross-cutting test that each of the five pillars must pass on its own terms, rather than a separate workstream competing with them for budget, staff, and attention. The distinction is structural, not rhetorical. A sixth pillar could be sequenced last, funded separately, or treated as an adjunct to the work that determines whether disparities actually close. An imperative has none of those escape hatches: each pillar's deliverable carries an equity condition it must satisfy. Does the mandate close disparities or widen them (Policy)? Does the data make disparities visible, or bury them in averages (Technology)? Do the incentives reward serving the hardest-to-reach populations, or penalize it (Economics)? Effective — but effective for whom (Clinical)? Executable everywhere, including in rural and under-resourced settings (Operations)?

## The HTR Ecosystem: How the Book, the Platform, and the Academy Work Together

This book is one of three integrated components of the Health Transformation Review ecosystem. The framework covered in this book is not only an argument to be followed — it is a system to be operated, and the platform (<https://healthtransformationreview.org>) and Academy (<https://healthtransformationreview.org/academy>) are where you operate it. Understanding how the three fit together will change how you read what follows.

| Component           | What it is                                                                                                                                                                                                                            | How it connects to the book                                                                                          |
|---------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------|
| <b>The Book</b>     | Transforming Healthcare — the framework, the dependency logic, the Vermont evidence, and national comparisons. Read online at <a href="https://healthtransformationreview.org/book">https://healthtransformationreview.org/book</a> . | The intellectual foundation. Everything else is an interactive expression of it.                                     |
| <b>The Platform</b> | Five pillar hubs, Research Lab with ~40 interactive tools, AI Analyst, 50-state dashboard, and daily Wire feed.                                                                                                                       | Every argument here that can be modeled is modeled there. Each chapter ends with an 'Explore on the Platform' guide. |
| <b>The Academy</b>  | Structured executive-education layer of courses, tracks, and lessons. Five Pillars, One Imperative is the companion course.                                                                                                           | Each chapter ends with a 'Go Deeper — Academy' guide naming specific hands-on lessons.                               |

## Chapter 1: The Five-Pillar Framework and the Execution Sequence

*Healthcare transformation is a system problem: five pillars — Policy, Technology, Economics, Clinical, and Operations — connected by nine dependency relationships that determine whether reform succeeds or fails. This chapter opens with the failure that proves the point — OneCare Vermont's collapse — then defines the five pillars, maps their interdependencies, shows what cascades when any one is missing, and turns to the question those dependencies force: in what order must the pillars be built? Vermont provides both the proof of concept and the canonical failure case — OneCare Vermont's collapse is the most instructive sequencing autopsy in recent American health policy.*

### 1.1 The Problem with Single-Pillar Thinking

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

**Every pillar you leave out becomes the constraint that defeats the ones you built.**

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The five pillars are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the five pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

### 1.2 The OneCare Failure: A Sequencing Autopsy

The clearest evidence for that argument is a failure that has already happened. Before defining the five pillars, it is worth examining OneCare Vermont's collapse in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more

useful than simply noting that the ACO wound down after roughly twelve years, founded in 2013 and closing at the end of 2025; the federal All-Payer ACO Model agreement that defined its most consequential years ran about eight of those years before its own scheduled sunset.

### 1.2.1 Sequencing Failure 1: Economics Without Policy Foundation

The most fundamental flaw in OneCare Vermont's design was attempting to operate a complex, population-level payment model—requiring system-wide accountability—without a policy foundation capable of enforcing full participation. Vermont lacked the regulatory authority to compel private commercial health plans into the model. This regulatory limit had a specific legal origin. In 2016, the U.S. Supreme Court ruled 6–2 in *Gobeille v. Liberty Mutual Insurance Co.* that ERISA preempts state laws requiring self-insured employer health plans to report claims data into state all-payer databases. That legal limitation became a concrete failure on January 1, 2023, when Blue Cross Blue Shield of Vermont—the state's largest private insurer—exercised its legal right to opt out and refused to renew its OneCare contract. In a single step, Blue Cross removed approximately 93,000 enrollees, stripping away roughly one-third of OneCare's total population base.

Act 68's mandatory architecture is the direct statutory response to this failure. Reference-based pricing and global budgets are not optional — every Vermont hospital is in. But a mandate is only the first of five conditions, and running this one decision through all five is the fastest way to see what the framework does.

#### WORKED EXAMPLE — MANDATORY GLOBAL BUDGETS THROUGH ALL FIVE PILLARS

Policy (permissible?): YES. Act 68 makes global budgets binding on every Vermont hospital — the mandatory lever federal law leaves Vermont.

Technology (possible?): NOT YET. Only if VHCURES and AHS-GMCMCB analytics can set and monitor each hospital's budget; without the data, a global budget is a number no one can manage.

Economics (sustainable?): YES, BY DESIGN. The budget inverts the incentive — every avoided admission now improves margin instead of cutting revenue.

Clinical (effective?): ONLY IF DELIVERY FOLLOWS. The margin is real only when Blueprint care teams actually prevent the admissions the budget rewards.

Operations (executable?): NOT YET. Running budgets at all 14 hospitals requires project-management capacity Vermont is still building.

### 1.2.2 Sequencing Failure 2: Economics Without Technology — The 'Managing Blind' Failure Mode

OneCare Vermont's second structural flaw was attempting to manage a complex, population-level Total Cost of Care model involving real financial risk without the data infrastructure required to execute it.

Managing value-based risk requires two distinct types of data integrated in near-real time: Claims Data (what care was delivered and at what cost) and Clinical Data (diagnoses, lab values, medications, reflecting patient health status in real time). Neither dataset alone is sufficient. Claims data without clinical data creates a financial lag; clinical data without claims data provides no visibility into total spending.

OneCare assumed financial risk without combining these feeds at mature scale. A state audit revealed the utilization signature of a population managed reactively rather than proactively—patients attributed to OneCare visited the emergency department 36% more frequently than peer ACOs nationally, while recording 18% fewer primary care visits. This is the 'Managing Blind' failure mode.

### 1.3 The Five Pillars and the Equity Imperative

Transformation has five moving parts you build and run — and one test they all must pass. The five pillars are capability domains: Policy, Technology, Economics, Clinical, and Operations. Each is a subsystem you can staff, fund, and hold accountable, and each answers a different question about whether transformation will actually work.

- **Policy:** The legislative and regulatory environment that creates the mandatory architecture. Acts 167 and 68 and the Statewide Strategic Plan mandate convert voluntary reform into statutory obligation.
- **Technology:** The data infrastructure, health information exchange, analytical platforms, and clinical technologies. VHCURES, VITL/VHIE, the AHS-GMCB analytics capability, and the Vermont CIN are Technology pillar investments.
- **Economics:** The payment and financial architecture. Reference-based pricing (FY2027), mandatory global hospital budgets (FY2028-2030), and total-cost-of-care accountability are Vermont's Economics pillar interventions.
- **Clinical:** The care delivery models, quality measurement frameworks, and patient care programs. Blueprint for Health (PCMH), Collaborative Care Model (CoCM) behavioral health integration, and CCBHC expansion are Clinical pillar interventions.
- **Operations:** The organizational capacity, management infrastructure, and administrative simplification. The 14-hospital transformation planning process, RHRC technical assistance, AHS HSA Coordinator model, and EMS regionalization are Operations pillar investments.
- **Equity Imperative:** The cross-cutting justice test applied to every pillar. Geographic equity across Vermont's 14 HSAs, SDOH investment through Blueprint HRSN screening, and health-equity metrics mandated in Acts 167 and 68 hold each pillar accountable.

### 1.4 The Architecture of Interdependency: The Dependency Matrix

The five pillars are linked by nine directed dependencies. Most set the build order; two run backward as feedback once the system is live. Reading those nine relationships accurately is the core analytical skill this book develops.

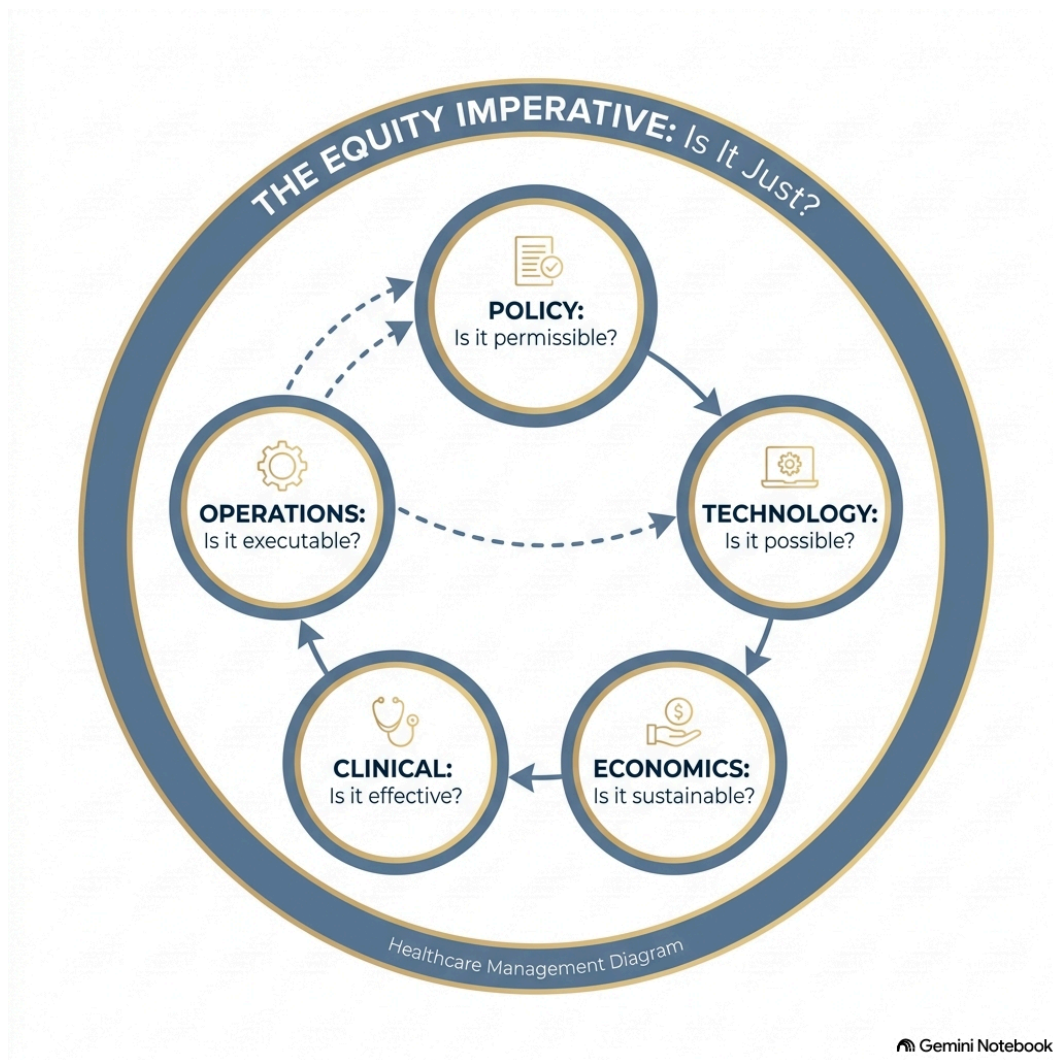


Figure 1.3 Architecture Diagram — The Five-Pillar Framework & Equity Imperative Dependency Cycle.

### The 9 Directed Dependencies Matrix

| FROM ↓ / TO →     | Policy | Technology                             | Economics                             | Clinical                                 | Operations                                 |
|-------------------|--------|----------------------------------------|---------------------------------------|------------------------------------------|--------------------------------------------|
| <b>Policy</b>     | —      | ENABLES: funds & authorizes data build | ENABLES: makes payment reform binding |                                          | DRIVES: statutory deadlines force capacity |
| <b>Technology</b> |        | —                                      | ENABLES: analytics run payment model  | ENABLES: data enables population health  |                                            |
| <b>Economics</b>  |        |                                        | —                                     | DRIVES: incentives reshape care delivery |                                            |
| <b>Clinical</b>   |        |                                        |                                       | —                                        | REQUIRES: care models need execution infra |

|            |                                    |                                          |   |
|------------|------------------------------------|------------------------------------------|---|
| Operations | INFORMS ⇄: data<br>refines mandate | RUNS ⇄: workforce<br>runs infrastructure | — |
|------------|------------------------------------|------------------------------------------|---|

EDITORIAL CLARIFICATION — CURRENCY FLOWS & STAGGERED PHASING

Capital Grants vs. Operating Liquidity (Economics → Operations): Transformation capital grants (e.g., the \$195M/year Rural Health Transformation Program award) are issued by Policy via statutory appropriation. The Economics pillar does not issue capital; it issues incentive consequences. Once live, however, Economics provides operating margin retention that funds ongoing hospital operations.

Economics-as-Design vs. Economics-as-Management: Designing global budget formulas and reference-based pricing rules happens in parallel during Stage 2 (Technology build). However, Economics-as-Management — actually operating global budgets and holding providers to financial risk — is strictly gated behind Stage 2 Technology analytics deployment.

1.5 The Failure Cascade: What Happens When a Pillar Is Missing

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed.

| Pillar Removed | Immediate Failure                                                           | Second-Order Failure                                                      | System-Level Outcome                                                        |
|----------------|-----------------------------------------------------------------------------|---------------------------------------------------------------------------|-----------------------------------------------------------------------------|
| Policy         | Payment reform remains voluntary; high-cost actors opt out.                 | Clinical programs run without financial alignment; tech built but unused. | Incremental improvement among willing; hospital financial crisis continues. |
| Technology     | VBC contracts signed but not managed; attribution errors undetected.        | Contracts produce unexpected losses; care remains reactive.               | Providers exit VBC; payment reform stalls; equity gaps widen invisibly.     |
| Economics      | Clinical programs continue under fee-for-service volume incentives.         | Primary care investment produces outcomes but cannot capture ROI.         | Quality rises at margin; costs keep rising; fragility deepens.              |
| Clinical       | Payment reform changes incentives but has nothing to incentivize.           | Operations manages financial pressure through service cuts.               | Hospital financial crisis accelerates under global budget pressure.         |
| Operations     | Statutory mandates exist on paper; platforms procured; nothing implemented. | Policy accountability fails; deadlines slip; strategic plans filed.       | Transformation documents accumulate; deficit trajectory continues.          |

1.11 The Five Stages: Resolved

The correct execution sequence is: Policy → Technology → Economics → Clinical → Operations.

| Stage | Pillar | Key Action & Logic                                             | Vermont Anchor                        |
|-------|--------|----------------------------------------------------------------|---------------------------------------|
| 1     | Policy | Establish enforceable authority, mandate participation, secure | Acts 167, 51, 68; \$195M/yr RHT award |

|   |            |                                                                                                      |                                                                                |
|---|------------|------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------|
|   |            | transformation capital. Prevents opt-outs.                                                           |                                                                                |
| 2 | Technology | Build integrated clinical + claims data infrastructure (real-time) before financial risk is assumed. | VHCURES; VITL/VHIE; AHS-GMCB analytics; Vermont CIN                            |
| 3 | Economics  | Deploy global budgets and RBP onto a visible system. Incentive architecture restructure.             | Global budgets (Act 68); RBP at ≤200% Medicare; pre-cap EAST baseline (\$138M) |
| 4 | Clinical   | Redesign care delivery on aligned incentives. Realize ROI through population health management.      | PCMH transformation; CoCM; CCBHC expansion; Blueprint 5.8:1 ROI                |
| 5 | Operations | Close the administrative cost gap (\$1,303/discharge premium) and execute programs.                  | RHRC methodology; shared services; 14-hospital transformation planning         |

### 1.14 Vermont's Implementation Timeline: Reading the Sequence in Statutory Deadlines

Reading Vermont's statutory timeline through the five-pillar sequence reveals both its correct sequencing decisions and its critical vulnerabilities as of Fall 2026.

- **Policy (Completed 2022–2025):** Acts 167, 51, and 68 established the mandatory statutory architecture. \$195M/year RHT award secures capital. Vermont formally withdrew from AHEAD in July 2026 after CMS renegotiation capped the EAST Fund, proving the state statutory framework remains absolute and fully passable on state authority alone.
- **Technology (In Progress 2025–2026 - Critical Bottleneck):** AHS-GMCB analytics capability being stood up, Vermont CIN under development, VHCURES attribution modeling in progress. This is the primary gate that must open before Act 68's hospital global budgets go live in FY2028.
- **Economics (Staged 2027–2028):** Mandatory RBP begins FY2027; mandatory non-CAH hospital global budgets take effect FY2028 (Act 68 statutory mandate).
- **Clinical & Operations (Parallel & Building 2025–2028):** Blueprint PCMH expansion, CCBHC certifications, CoCM deployment, and RHRC hospital transformation planning proceed in parallel.

### 1.15 EXPLORE ON THE PLATFORM

Manipulate the execution-sequence argument directly using the interactive tools on the HTR platform:

- **Five-Pillar Map (<https://healthtransformationreview.org/about/framework>):** Trace the 9 dependency relationships and highlight what each pillar enables and requires.
- **HTR Simulator (<https://healthtransformationreview.org/htr-simulator>):** Score an organization or state across all five pillars and observe downstream penalty cascades.

- **Transformation Friction Index** (<https://healthtransformationreview.org/transformation-friction-index>): Identify your own Technology gate bottleneck before it becomes a financial risk.
- **Impact Simulation** (<https://healthtransformationreview.org/impact-simulation>): Model how a shortfall in one pillar propagates through the other four over time.

## 1.16 GO DEEPER — ACADEMY

Work Track 1 ('Foundations: The Framework') of the Five Pillars, One Imperative course at <https://healthtransformationreview.org/academy/tracks/five-pillars-one-imperative> to master dependency logic and the OneCare autopsy.

## 1.17 Implications for You

**If you are a Vermont hospital executive:** The most important action you can take before Act 68's global budgets take effect in FY2028 is completing HCC gap closure for your full Medicare-attributed population. This single Technology pillar investment improves risk adjustment accuracy, produces a more defensible global budget benchmark, and prevents the 'Managing Blind' failure mode in your first Act 68 global budget performance year. Do not wait for the AHS-GMCB analytics capability to complete this work. Start with VHCURES direct access and your current analytics resources now.

**If you are an AHS or GMCB official:** The Technology gate is your critical bottleneck, and it has a hard deadline — analytics capability must be operational before Act 68's FY2028 global budgets take effect. The three-dependency convergence — global-budget management, Statewide Strategic Plan measurement, CCBHC risk stratification — means that a delay in the analytics capability is a compound failure across multiple statutory obligations.

**If you are a Vermont legislator:** The sequencing framework translates directly into the right oversight questions to ask AHS and GMCB at every reporting interval: Is the Technology gate open — analytics actually operating, not merely procured — before the Economics gate goes live? Is the RBP methodology being designed with equity constraints from the beginning? Is the RHRC engagement building hospital-level Operations capacity ahead of Act 68's global budget accountability requirements?

**If you are a national health policy professional:** Vermont's experience provides a detailed real-world test of whether a mandatory state payment architecture (Act 68) can improve outcomes on state authority alone, after withdrawing from the federal AHEAD CMMI model when federal terms collapsed. Watch Vermont's Act 68 implementation closely.

## 1.18 Key Concepts in This Chapter

- **Dependency logic** — The principle that some pillar investments cannot produce results until upstream enabling investments are in place. Determines the execution sequence.
- **Execution sequence** — The order in which the five pillars must be built: Policy -> Technology -> Economics -> Clinical -> Operations.
- **Critical path gate** — A prerequisite investment that must be in place before a downstream pillar investment can be productive.

- **'Managing Blind' failure mode** — Assuming downside financial risk under a value-based payment model without the analytics infrastructure to track total cost of care in real time.
- **Voluntary vs. mandatory architecture** — The distinction between reform programs actors can opt out of and those they cannot. Act 68 establishes mandatory RBP and global budgets.
- **Economics-as-design vs. Economics-as-management** — Design (defining payment architecture) can proceed in parallel during Stage 2. Management (operating global budgets) requires the Stage 2 technology substrate to be operational first.